Cross-Border Medico-Legal Strategy: Pursuing Psychiatric Negligence, Data Rights, and Institutional Accountability from the United Kingdom

1. Executive Summary and Strategic Overview

This report provides a comprehensive legal, medical, and strategic analysis for a dual British-Irish national currently domiciled in London, United Kingdom, who is seeking redress for alleged severe medical malpractice, police misconduct, and institutional administrative failures originating in the Republic of Ireland. The subject matter concerns a complex intersection of cross-border jurisdictional law post-Brexit, biological versus trauma-informed psychiatric paradigms, and the enforcement of data protection rights against recalcitrant state bodies.

The core grievance centers on the clinical conduct of Dr. Colm McDonald, a Professor of Psychiatry at the University of Galway and a consultant within the Health Service Executive (HSE) West. The claimant alleges that Dr. McDonald, adhering to a rigid biological model of mental illness, misdiagnosed a clear history of complex trauma as a series of biological mood disorders—specifically Drug-Induced Psychosis, Bipolar Disorder, Bipolar Affective Disorder, and finally Schizoaffective Disorder. This diagnostic trajectory, characterized by the claimant as a "spectrum" of biological labels "for genetic reasons that were never found," stands in direct contradiction to a subsequent diagnosis of Complex Post-Traumatic Stress Disorder (CPTSD) made by an independent psychiatrist. The claimant asserts that the resulting pharmacological regimen, consisting of "overdoses of neuroleptics," has caused permanent iatrogenic injury to the brain, alongside physical injuries to the spine and jaw sustained during interactions with the Irish police (An Garda Síochána).

A critical dimension of this case is the claimant's inability to return to the Irish jurisdiction due to credible fears for their physical safety, citing threats from a "militia" associated with the psychiatrist and previous severe injuries inflicted by the police. Consequently, the claimant requires a legal strategy that can be executed primarily from the United Kingdom, leveraging their British nationality and residence to force accountability upon Irish institutions that have allegedly "circled the wagons."

The analysis herein confirms that the claimant possesses a viable pathway to litigate these matters from London. The "Tort Gateway" established by the UK Supreme Court in *Brownlie v FS Cairo (Nile Plaza) LLC* allows for claims to be heard in English courts if the "damage" (ongoing suffering, financial loss, medical treatment) is sustained within the jurisdiction, even if the initial negligent act occurred abroad.[1] Furthermore, the impasse regarding the release of medical records—where Irish authorities cite "mental health risk" to withhold data—can be

legally circumvented using the "nominated health professional" mechanism under the *Data Protection Act 2018 (Access Modification) (Health) Regulations 2022*.[2]
This report is structured to provide an exhaustive, step-by-step roadmap. It details how to establish jurisdiction in the English High Court, how to scientifically deconstruct the psychiatrist's biological diagnosis using his own research against him, and how to utilize GDPR mechanisms to unfreeze university records that are currently blocking the claimant's academic progression.

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2. Jurisdictional Analysis: Litigating Irish Negligence in English Courts

The primary strategic hurdle in this case is the "cross-border" nature of the dispute. The alleged torts (medical negligence, police assault) occurred in Galway, Ireland, while the claimant resides in London. Traditionally, under the Brussels I Recast Regulation, a defendant domiciled in an EU Member State (Ireland) must be sued in that state. However, the United Kingdom's withdrawal from the European Union has fundamentally altered the rules of private international law, creating distinct advantages for British nationals injured abroad who return to the UK.

2.1 The Post-Brexit Legal Landscape and the Common Law Reversion

Since 1 January 2021, the Brussels regime no longer applies to proceedings instituted in the UK courts regarding defendants in EU states (unless proceedings were pending before that date). Consequently, jurisdiction is now governed by the common law rules of England and Wales.[4] This reversion to common law allows the English courts a wider discretion to accept jurisdiction over foreign torts than was permissible under the strict European regime.
To sue an Irish-domiciled defendant (Dr. McDonald or the HSE) in London, the claimant must obtain permission from the court to serve the claim form out of the jurisdiction. This requires satisfying three tests:

1. **Serious Issue to be Tried:** There must be a real prospect of success on the merits of the claim.
2. **The Jurisdictional Gateway:** The claim must fall within one of the "gateways" set out in Practice Direction 6B of the Civil Procedure Rules (CPR).
3. **Forum Conveniens:** England must be the proper place to bring the claim.

2.2 The "Damage Gateway" and the *Brownlie* Precedent

The most critical legal development for this claimant is the Supreme Court judgment in *FS Cairo (Nile Plaza) LLC v Brownlie* UKSC 45. This case redefined the interpretation of the "tort gateway" found in CPR PD 6B para 3.1(9)(a), which allows a claim to be brought in England if "damage was sustained... within the jurisdiction."

2.2.1 Expanding the Definition of "Damage"

Prior to *Brownlie*, there was significant legal uncertainty regarding whether "damage" referred strictly to the *direct* effect of the tort (e.g., the initial misdiagnosis or the administration of the drug in Galway) or if it extended to *indirect* or *consequential* damage (e.g., the ongoing pain, financial loss, and psychiatric distress suffered upon returning to England).
The Supreme Court, by a 4-1 majority, adopted the wider interpretation. Lord Lloyd-Jones held that "damage" extends to the actionable harm caused by the tortious act, including **significant consequential damage**. This ruling explicitly allows UK residents injured abroad to rely on their ongoing suffering and financial losses in the UK to found jurisdiction in the English courts.[1]

2.2.2 Application to the Claimant's Circumstances

The claimant alleges "permanent injury to my brain" and "injuries to my spine and jaw." While the *infliction* of these injuries occurred in Ireland, the *sustenance* of the damage—the daily experience of pain, the cognitive impairment, the need for ongoing medical care, and the inability to work or study—is occurring in London.

- **Ongoing Psychiatric Injury:** The management of the claimant's Complex PTSD, which requires specialized trauma therapy available in London (and allegedly denied in Galway), constitutes ongoing damage.
- **Financial Loss:** If the claimant is incurring costs for therapy in London, or has lost earnings in the UK due to the incapacitation caused by the brain injury, this is financial damage sustained within the jurisdiction.
- **The "Brownlie" Argument:** Therefore, the claimant has a statutory right to request that the English High Court hear the case. The "damage" is not a historical event in Ireland; it is a present reality in London.

2.3 The *Forum Non Conveniens* Battleground

Establishing a gateway is necessary but not sufficient. The court must also be persuaded that England is the *forum conveniens*—the appropriate forum for the trial. The defendants (Dr. McDonald, HSE, University of Galway) will inevitably apply for a stay of proceedings, arguing that Ireland is the "natural forum" because the events, witnesses, and applicable law are Irish.[6]
To defeat this challenge, the claimant must demonstrate that **substantial justice cannot be done in the foreign forum (Ireland).** This is a high threshold, but the specific facts of this case provide a compelling argument.

2.3.1 The "Safety and Fair Trial" Argument

The claimant explicitly states: *"I am not safe to return to Ireland because of him and because of his own little militia... the Irish police forces severely injured me."* [User Query].

- **Legal Significance:** Under the *Spiliada* principles (which govern *forum non conveniens*), the court looks at practical justice. If a claimant cannot physically attend the trial in the foreign jurisdiction due to a well-founded fear for their safety, or if they risk further persecution by state agents (the police) upon return, the English court may refuse to stay the proceedings.
- **Evidence Requirement:** The claimant must provide the UK legal team with documented

evidence of the police assault (medical reports of spine/jaw injury), any threats received, and the specific reasons why the "militia" poses a threat. The existence of a "closed shop" or systemic bias in the Galway region (where the defendant is a prominent Professor and Consultant) can also be argued as an impediment to a fair trial locally.

2.3.2 The Medical Nexus in London

The claimant's current medical reality is anchored in London. The diagnosis of CPTSD—which refutes Dr. McDonald's diagnosis—was made by "another psychiatrist" (presumably in the UK context or one whose evidence is accessible).

- **Expert Witnesses:** The trial will turn on the "battle of experts." The claimant will rely on UK-based trauma specialists to testify that Dr. McDonald's biological focus and failure to consider trauma constituted negligence. These experts are in London. Litigating in London facilitates their attendance and reduces costs compared to flying them to Galway.[6]

2.3.3 The "Administrative Justice" Argument

The claimant notes that the Irish Ombudsman and Mental Health Commission have "claimed that they can't get involved in any clinical case." This admission of a regulatory vacuum in Ireland strengthens the argument that the English High Court is the only venue capable of providing a remedy. If the local remedies have been exhausted or are statutorily barred from investigating the core clinical issue, the claimant can argue that forcing them back to Ireland is a denial of justice.

2.4 Applicable Law: The *Rome II* Regulation

It is imperative to understand that even if the case is heard in London, the English judge will likely apply **Irish Law** to the substantive question of negligence. The UK has retained the *Rome II Regulation* (Regulation (EC) No 864/2007) post-Brexit. Article 4(1) of Rome II states that the law applicable to a tort shall be the law of the country in which the damage occurred.[4]

- **Implication:** The English court will apply the Irish legal test for medical negligence (the *Dunne* principles) rather than the English *Bolam* test.
- **Strategic Advantage:** This is not a disadvantage. The *Dunne* test is robust. The advantage lies in having an **English Judge** applying that law, free from local Irish institutional influence, bias, or the "militia" pressures the claimant fears. Furthermore, English procedural rules on disclosure and expert evidence are rigorous and may force the release of data that Irish bodies are withholding.

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3. Medical Malpractice Analysis: The Biological vs. Trauma Paradigm

The crux of the clinical negligence claim is the allegation that Dr. McDonald, driven by a rigid "biological psychiatry" ideology, ignored a clear history of trauma to diagnose a "genetic" mood disorder, thereby causing iatrogenic harm through unnecessary neuroleptic treatment.

3.1 The Defendant's Profile: A Career in Biological Determinism

Dr. Colm McDonald is not merely a clinician; he is a high-ranking academic with a specific research agenda. He is the Director of the Clinical Neuroimaging Laboratory at the University of Galway.[7] His research output is prolifically focused on the **biological mechanisms** of mental illness.

- **Neuroimaging Bias:** His publications heavily feature MRI studies mapping brain structure to illness, such as *"Subcortical volumetric abnormalities in bipolar disorder"* [8] and *"Cortical abnormalities in bipolar disorder"*.[9]
- **Genetic Focus:** He is a key contributor to the ENIGMA consortium, which seeks to identify genetic and neurobiological markers for schizophrenia and bipolar disorder.[10]
- **The "Genetic Reasons Never Found":** The claimant accurately critiques this approach. Despite decades of research, no singular genetic or structural cause for these disorders has been definitively found to exist in isolation from environmental factors. By prioritizing a theoretical biological model over the patient's actual history of "severe violence," the claimant can argue that Dr. McDonald allowed his academic bias to cloud his clinical judgment.

3.2 Diagnostic Negligence: Bipolar Spectrum vs. CPTSD

The claimant describes a diagnostic progression: *Drug Induced Psychosis -> Bipolar Disorder -> Bipolar Affective Disorder -> Schizoaffective Disorder*. This "diagnostic creep" is characteristic of a clinician attempting to fit a complex presentation into a biological box.

3.2.1 The Divergence

- **Bipolar/Schizoaffective Disorder:** These are classified as endogenous mood/psychotic disorders, treated primarily with medication (lithium, valproate, antipsychotics) to stabilize "chemical imbalances".[11]
- **Complex PTSD (CPTSD):** This condition, formalized in ICD-11 and recognized by the UK's NICE guidelines, arises from prolonged, inescapable trauma (e.g., childhood abuse, domestic violence). It presents with emotional dysregulation, negative self-concept, and relational difficulties.[12]
- **The Overlap and Error:** Research confirms that CPTSD is frequently misdiagnosed as Bipolar Disorder because both involve mood swings and irritability.[14] However, the *treatment* is radically different. Bipolar treatment (medication) does not resolve trauma; trauma treatment (psychotherapy) does.

3.2.2 The "Super Sesh" Admission

The claimant notes a critical piece of evidence: Dr. McDonald stated his opinion that the Bipolar Disorder was "also comorbid with complex post-traumatic stress disorder" (a "super

sesh" or superimposition).

- **Legal Significance:** This is a potential "smoking gun." If the psychiatrist *admitted* the presence of CPTSD but failed to treat it, or continued to treat the patient *only* as a biological case despite knowing of the trauma, this is negligence. It demonstrates a failure to provide holistic care. Furthermore, if he acknowledged CPTSD, he cannot later claim the symptoms were *solely* due to a genetic mood disorder. His own admission contradicts his "working diagnosis."

3.3 Causation: Iatrogenic Brain Injury

The claimant alleges "permanent injury to my brain" caused by "overdoses of neuroleptics."

- **The Mechanism:** Antipsychotic medications (neuroleptics), particularly at high doses or in polypharmacy (using multiple drugs), are known to have significant side effects. Long-term use has been associated in some studies with **brain volume loss** (cortical thinning) and dopamine receptor supersensitivity (causing tardive dyskinesia).[15]
- **Turning the Research Around:** Ironically, Dr. McDonald's own field of research (neuroimaging) can be used to prove the injury. If the claimant can obtain MRI scans, a neuroradiologist could potentially identify structural changes consistent with drug toxicity rather than the alleged illness.
- **Spinal/Jaw Injury:** The claimant also links these injuries to the psychiatrist and the "militia." If these injuries occurred during involuntary committal or restraint procedures initiated by the psychiatrist, they fall under the ambit of medical assault or battery, as well as negligence.

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4. Data Protection and Information Rights: Breaking the Deadlock

The claimant is in a "catch-22" where Irish authorities refuse to release medical records, citing the "Risk of Harm" exemption. This is a standard defensive tactic in psychiatric cases, but it is not absolute. As a British national resident in the UK, the claimant has specific levers to pull.

4.1 The "Serious Harm" Exemption and the UK Bypass

Under Section 60(5) of the Irish Data Protection Act 2018, access to health data can be restricted if it would likely cause "serious harm to the physical or mental health of the data subject".[2] The HSE is using this to block the claimant.

However, the **Data Protection Act 2018 (Access Modification) (Health) Regulations 2022** (S.I. No. 121 of 2022) mandates that if direct access is refused, the data controller **must offer to release the data to a medical practitioner nominated by the individual**.[3]

Strategic Action:

The claimant must formally nominate a **UK-based General Practitioner (GP)** or private psychiatrist to receive the records.

- **Why this works:** The Irish HSE cannot credibly argue that releasing records to a

qualified UK doctor will cause harm. Once the records are in the hands of the UK doctor, they are subject to UK GDPR and the professional judgment of that doctor. If the UK doctor (who supports the CPTSD diagnosis) assesses the claimant as stable, they can release the records to the claimant.

4.2 Demanding Digital Copies (Article 15(3) GDPR)

The claimant states the HSE "refused to give me digital notes." This is a direct violation of the GDPR.

- **The Law:** Article 15(3) of the GDPR states: *"Where the data subject makes the request by electronic means, and unless otherwise requested by the data subject, the information shall be provided in a **commonly used electronic form**."*.[17]
- **The Argument:** The HSE cannot force a paper release on a data subject who is resident in another jurisdiction (UK) and for whom travelling to collect paper records is unsafe. The refusal to provide digital copies is an obstruction of the right of access. This specific failure should be the subject of a complaint to the **Data Protection Commission (DPC)**.

4.3 Rectification of Student Records (Article 16 GDPR)

The claimant wants the diagnosis "frozen" in the university record to be removed.

- **The Challenge:** A medical opinion, if genuinely held at the time, is rarely considered "inaccurate" data that can be deleted under Article 16.[19]
- **The Solution: The "Supplementary Statement":** Article 16 grants the right to have incomplete personal data *completed*, including by means of providing a **supplementary statement**.[19]
- **Execution:** The claimant should draft a formal legal statement titled *"Supplementary Statement of Medical Fact pursuant to Article 16 GDPR."* This document should detail the new CPTSD diagnosis, the rejection of the Bipolar diagnosis, and the allegation of negligence. The University is legally obliged to place this statement **on the file** alongside the disputed diagnosis. This ensures that any future decision-maker (e.g., in a Masters application) sees the dispute immediately and cannot rely solely on the "frozen" diagnosis.

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5. Administrative Justice: The University and The State

5.1 The University Visitor: The Final Appeal

The claimant describes the Dean of Students as having "decided that I have no rights." In the structure of the National University of Ireland (NUI), of which Galway is a constituent, the **Visitor** is the supreme appellate authority for internal disputes.[21]

- **Role of the Visitor:** The Visitor (often a senior judge) has the power to review the interpretation of University Statutes. If the Dean of Students has acted unfairly or

blocked academic progression based on a contested medical opinion (where the professor has a conflict of interest as both clinician and academic), this is a matter for the Visitor.
- **Action:** The claimant should petition the Visitor, arguing that the University's reliance on a disputed diagnosis to deny educational access is a breach of the University Charter and the principles of natural justice.

5.2 The Ombudsman for Higher Education

If the Visitor process stalls, the **Office of the Ombudsman** can investigate "maladministration" by the University.[22]

- **Maladministration:** This includes unfair decision-making, failure to consider relevant evidence (the UK diagnosis), and delay. The Ombudsman cannot overturn a clinical diagnosis, but can rule that the *administrative use* of that diagnosis to penalize a student is unfair.

5.3 Police Misconduct: Fiosrú (The New GSOC)

The claimant alleges "severe injury" by the Irish police and fears a "militia." The Garda Síochána Ombudsman Commission (GSOC) has been reconstituted as **Fiosrú (Office of the Police Ombudsman)** under the *Policing, Security and Community Safety Act 2024*.[23]

- **Reporting from the UK:** Complaints can be submitted online from abroad.
- **Public Interest Investigation:** Given the allegation of "militia" involvement and severe injury, the claimant should request a **public interest investigation** (formerly Section 102 referral). This removes the investigation from local Garda management. The claimant should emphasize that their displacement to the UK was a direct result of this police misconduct, justifying any delay in reporting.

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6. Strategic Roadmap and Conclusion

The claimant is currently in a position of asymmetry: the Irish institutions hold the data and the territory. To shift the balance of power, the claimant must execute a strategy that leverages **UK jurisdiction**, **European data rights**, and **independent medical evidence**.

Phase 1: Secure the Evidence (Weeks 1-4)

1. **Nominate UK GP:** Identify a sympathetic UK doctor and formally nominate them to the HSE/University to receive the medical records, citing the *Health Regulations 2022*.
2. **GDPR Strike:** Issue a new Subject Access Request demanding **digital copies** under Article 15(3) and attaching the nomination. Simultaneously, issue the Article 16 **Supplementary Statement** to the University to "flag" the student record.
3. **Medical Report:** Commission a comprehensive psychiatric report in London confirming CPTSD and explicitly ruling out Bipolar/Schizoaffective Disorder. This report is the foundational document for all subsequent legal actions.

Phase 2: Establish Jurisdiction (Months 2-6)

1. **Letter of Claim:** Instruct UK solicitors (specialists in cross-border injury) to issue a Letter of Claim to Dr. McDonald and the HSE.
2. **The *Brownlie* Argument:** The solicitors must assert jurisdiction in the English High Court based on the "Damage Gateway"—that the claimant is suffering the pain, financial loss, and medical consequences of the negligence *in London*.
3. **Fight *Forum Non Conveniens*:** Prepare a detailed witness statement regarding the "safety" threats in Ireland (militia/police) to prove that a fair trial cannot be held in Galway. This is the key to keeping the case in the UK.

Phase 3: Regulatory Pressure (Ongoing)

1. **Medical Council:** File a formal complaint against Dr. McDonald for "Poor Professional Performance" regarding the failure to take a trauma history. Use the UK expert report as evidence.
2. **Fiosrú:** Submit the police complaint online, attaching medical evidence of the spine/jaw injuries.
3. **Media:** Approach investigative platforms like *"The Ditch"* or *"Mad in Ireland"* with the narrative of a whistleblower/survivor forced into exile by a collusion of medical and police failures.

By shifting the battlefield to the UK courts and using the GDPR to bypass Irish obstructionism, the claimant can force the transparency and accountability that has so far been denied.

Tables of Reference

Table 1: Key Legal Mechanisms for the Claimant

Mechanism	Statute/Case Law	Purpose
Tort Gateway	*Brownlie v FS Cairo* (UKSC 45)	Allows suing Irish doctor in UK courts due to ongoing damage in London.
Data Bypass	*Health Regulations 2022* (S.I. 121)	Forces release of medical records to a nominated UK doctor despite "harm" claim.
Digital Right	*GDPR Article 15(3)*	Mandates release of digital notes, overcoming refusal of digital formats.
Record Flagging	*GDPR Article 16*	Inserts "Supplementary Statement" of CPTSD into frozen university files.
Police Complaint	*Policing Act 2024*	Investigates police assault via Fiosrú (independent body).

Table 2: Biological vs. Trauma Psychiatry (The Clinical Dispute)

Feature	Biological Model (Dr. McDonald)	Trauma Model (Claimant/NICE)
Cause of Illness	Genetic / Neurobiological / Structural	Environmental / Trauma / Abuse
Diagnosis	Bipolar / Schizoaffective Disorder	Complex PTSD (CPTSD)
Primary Treatment	Neuroleptics (Antipsychotics)	Trauma-Focused Therapy (CBT/EMDR)
Approach to Trauma	"Irrelevant" / Coincidental	The central causal factor
Outcome	"Permanent injury to brain" (Iatrogenic)	Potential for recovery / "Healable framework"

Disclaimer: *This report represents a high-level research and strategic analysis based on the information provided. It does not constitute formal legal advice. The claimant is strongly urged to engage specialist legal counsel in the UK to execute these steps.*

Works cited

1. Still got the Brexit Blues | 39 Essex Chambers, accessed February 17, 2026, https://www.39essex.com/information-hub/blog/still-got-brexit-blues
2. Health Data & Subject Access Requests: Important Changes under the Data Protection Act 2018 - WILLIAM FRY, accessed February 17, 2026, https://www.williamfry.com/knowledge/health-data-subject-access-requests-important-changes-under-the-data-protection-act-2018/
3. Changes in How Controllers Handle Health Data in Data Subject Access Requests, accessed February 17, 2026, https://www.mccannfitzgerald.com/knowledge/data-privacy-and-cyber-risk/changes-in-how-controllers-handle-health-data-in-data-subject-access-requests
4. Cross-border Litigation after Brexit - Keoghs, accessed February 17, 2026, https://www.keoghs.co.uk/keoghs-insight/cross-border-litigation-after-brexit
5. UK – EU Cross-Border Disputes: Where Should I Bring My Claim? - Lester Aldridge, accessed February 17, 2026, https://www.lesteraldridge.com/blog/litigation-recoveries/uk-eu-cross-border-disputes-where-claim/
6. Forum Non Conveniens - When Home Isn't Always Convenient - Baker Sterchi, accessed February 17, 2026, https://www.bakersterchi.com/forum-non-conveniens-when-home-isnt-always-convenient
7. Colm McDonald - Wikipedia, accessed February 17, 2026, https://en.wikipedia.org/wiki/Colm_McDonald
8. 2026 Colm McDonald: Medicine Researcher – H-Index, Publications & Awards,

accessed February 17, 2026, https://research.com/u/colm-mcdonald
9. Colm McDonald - Google Scholar, accessed February 17, 2026, https://scholar.google.com/citations?user=W38WX44AAAAJ&hl=en
10. Colm McDonald - University of Galway Research, accessed February 17, 2026, https://research.universityofgalway.ie/en/persons/colm-mcdonald/
11. Misdiagnosis: Is It Bipolar Disorder or Complex Post-Traumatic Stress Disorder. | CPTSDfoundation.org, accessed February 17, 2026, https://cptsdfoundation.org/2020/11/23/misdiagnosis-is-it-bipolar-disorder-or-complex-post-traumatic-stress-disorder/
12. Complex post-traumatic stress disorder (PTSD) - HSE, accessed February 17, 2026, https://www2.hse.ie/conditions/ptsd/complex-ptsd/
13. Post-traumatic stress disorder (PTSD) and Complex PTSD - UK Trauma Council, accessed February 17, 2026, https://uktraumacouncil.org/trauma/ptsd-and-complex-ptsd
14. Differentiating the symptoms of posttraumatic stress disorder and bipolar disorders in adults: Utilizing a trauma-informed assessment approach - PubMed, accessed February 17, 2026, https://pubmed.ncbi.nlm.nih.gov/32822516/
15. Neurobiological trait abnormalities in bipolar disorder - PubMed, accessed February 17, 2026, https://pubmed.ncbi.nlm.nih.gov/19455151/
16. Ireland: Employers can now process Data Subject Access Requests without advice of health service providers | Privacy Matters, accessed February 17, 2026, https://privacymatters.dlapiper.com/2022/06/ireland-employers-can-now-process-data-subject-access-requests-without-advice-of-health-service-providers/
17. Processing of Patient Personal Data: - Irish College of GPs, accessed February 17, 2026, https://www.irishcollegeofgps.ie/Portals/0/Explore%20the%20College/Practice%20Management/IT%20&%20E-Health/PM_IT_GDPR_Guidelines_v2.pdf
18. The Right of Access - Data Protection Commission, accessed February 17, 2026, http://www.dataprotection.ie/en/individuals/know-your-rights/right-access-information
19. Can I Use the GDPR to have my medical records amended or erased?, accessed February 17, 2026, https://www.dataprotection.ie/en/can-i-use-gdpr-have-my-medical-records-amended-or-erased
20. Data Protection - GDPR - University Obligations, accessed February 17, 2026, https://www.universityofgalway.ie/data-protection/gdpr/universityobligations/
21. Student Appeals - University College Dublin, accessed February 17, 2026, https://www.ucd.ie/secca/studentappeals/
22. The Ombudsman and complaints about Education services, accessed February 17, 2026, https://assets.ombudsman.ie/media/285397/236b1f0d-7896-4fc7-abaf-37b192977a3e.pdf
23. Submit a Complaint - Fiosrú, accessed February 17, 2026, https://www.fiosru.ie/complaints/submit-a-complaint/